

1. Administrative & Study Identification

Full Study Title: A Study to Investigate the Efficacy and Safety of Volrustomig ± Casdatifan vs Nivolumab + Ipilimumab as 1L Treatment for Advanced ccRCC **Official Title:** A Phase Ib/III Randomized, Multicenter, Global Study of Volrustomig Plus Casdatifan or Volrustomig Monotherapy Versus Nivolumab Plus Ipilimumab as First-line Treatment for Participants With Advanced Clear Cell Renal Cell Carcinoma (ccRCC) **Short Title/Acronym:** eVOLVE-RCC02 **Protocol Number:** D798NC00001 **NCT Number:** NCT07000149 **Posting ID:** 764043096871782188 **Sponsor / Company Name:** AstraZeneca (in collaboration with Arcus Biosciences) **Investigational Product:** Volrustomig (anti-PD-1/CTLA-4 bispecific antibody) ± Casdatifan (HIF-2α inhibitor) **Phase of Development:** Phase Ib/III (Phase ID: PHASE3) **Trial Status:** ACTIVE_NOT_RECRUITING (Active but not currently recruiting) **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: * **Phase Ib:** To determine the recommended Phase III dose (RP3D) of volrustomig in combination with casdatifan and assess safety and tolerability. * **Phase III:** To measure the efficacy (Progression-Free Survival [PFS] and Overall Survival [OS]) of volrustomig plus casdatifan versus standard-of-care (SOC) nivolumab plus ipilimumab. **Secondary Objectives:** To evaluate Objective Response Rate (ORR), Duration of Response (DoR), Disease Control Rate (DCR), Pharmacokinetics (PK), and immunogenicity.

2.2 Background & Rationale Clear cell renal cell carcinoma (ccRCC) is fundamentally driven by HIF-2α dysregulation. While the current first-line standard-of-care includes checkpoint inhibitors like nivolumab plus ipilimumab, long-term survival rates remain low for metastatic disease. This study evaluates a potentially first-in-class, TKI-sparing combination regimen to simultaneously stop multiple cancer-driving processes using an anti-PD-1/CTLA-4 bispecific antibody alongside a HIF-2α inhibitor.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Nivolumab + Ipilimumab) **Blinding:** Open-label **Randomization:** * **Phase Ib:** 1:1 (volrustomig Dose 1 vs Dose 2 + casdatifan) * **Phase III:** 1:1:1 (volrustomig + casdatifan vs

volrustomig monotherapy vs nivolumab + ipilimumab) **Phase**
Criteria: * **Criteria Type:** PHASE_REQUIREMENT * **Criteria Text:**
Trial must be in PHASE3

3.2 Planned Enrollment & Duration Targeted Enrollment: 1,116
participants **Number of Sites:** Multicenter, global trial **Estimated**
Study Start: 28-Jul-2025 **Estimated Primary Completion:** 21-
Feb-2030

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Histologically or cytologically confirmed RCC with clear cell component. 2. Advanced/metastatic RCC or recurrent disease that has not previously been treated with systemic therapy in the 1L setting. 3. Karnofsky Performance Status $\geq 70\%$. 4. At least one lesion that can be accurately measured at baseline as ≥ 10 mm in the longest diameter (except lymph nodes, which must have short axis ≥ 15 mm) with CT or MRI and is suitable for accurate repeated measurements. 5. Provision of acceptable tumor sample.

4.2 Exclusion Criteria 1. Prior systemic therapy for advanced/metastatic RCC. 2. History of leptomeningeal disease, spinal cord compression, or symptomatic brain metastases. 3. Medical history of severe chronic obstructive pulmonary disease (COPD). 4. Active or prior documented autoimmune or inflammatory disorders.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Casdatifan (oral, daily) + Volrustomig (IV infusion, at the dose determined in Phase Ib). The active comparator arm receives standard-of-care Nivolumab + Ipilimumab. **Route of Administration:** Intravenous (IV) for antibodies (Volrustomig, Nivolumab, Ipilimumab); Oral for Casdatifan. **Duration of Treatment:** Until disease progression, unacceptable toxicity, or withdrawal of consent.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care medications to manage side effects. **Prohibited:** Concurrent systemic anticancer therapies, experimental agents, live vaccines, and immunosuppressive therapies.

5.3 Reference Drug Details (Active Comparator Component) * **Drug Name:** ipilimumab (Preferred Name: ipilimumab) * **Trade Names:** Yervoy * **ATC Code:** L01FX04 * **RxNorm Code:** Not Available * **EMA URL:** https://www.ema.europa.eu/en/documents/product-information/yervoy-epar-product-information_en.pdf

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, Tumor Imaging (CT/MRI), Labs
Baseline	Day 0	Randomization, First Dose, PK/PD Sampling	Interim Every Cycle Dosing, Safety Labs, Tumor Imaging, Efficacy Assessment
End of Study	Progression/Tox	Final Vital Signs, Exit Interview, Safety Follow-up	

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: * **Phase Ib:** Number of participants with adverse events (AEs) and serious adverse events (SAEs) who received at least one dose of study treatment. * **Phase III:** Progression-Free Survival (PFS) defined as the time from randomization until radiological progression or death due to any cause. Overall Survival (OS) defined as time from randomization until the date of death due to any cause.

7.2 Secondary & Exploratory Endpoints * **Phase Ib:** Objective Response Rate (ORR), Duration of Response (DoR), and Progression-free Survival (PFS). * Disease Control Rate (DCR), Pharmacokinetics (PK parameters), and Immunogenicity.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Collection of clinical and laboratory AEs (focus on immune-related AEs, anemia, and hypoxia linked to HIF-2α inhibition). **Serious Adverse Events (SAEs):** Reported within 24 hours of site awareness. **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee (IDMC) will review safety and efficacy data across both study phases.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for efficacy endpoints (PFS, OS); Safety Set for all participants receiving at least one dose of study treatment. **Sample Size Justification:** Target enrollment of 1,116 appropriately powered to detect statistically significant improvements in PFS and OS for the experimental combinations versus SOC. **Handling of Missing Data:** Standard censoring rules for time-to-event endpoints (PFS, OS) per regulatory guidelines.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine onsite and remote monitoring visits to ensure data integrity and protocol adherence. **Audit Trail:**

Electronic Case Report Form (eCRF) locking and data clarification forms via standard quality audit mechanisms.

11. Study Identifiers & Governance

Primary ID: D798NC00001 **Registry Number:** NCT07000149 **Posting ID:** 764043096871782188 **Sponsor/Lead Organization:** AstraZeneca (Company: AstraZeneca) **Source Level:** 5

12. Clinical Framework (ccRCC Specific)

Disease Areas: Advanced Clear Cell Renal Cell Carcinoma **Name:** Advanced Clear Cell Renal Cell Carcinoma **Preferred Name:** Advanced Clear Cell Renal Cell Carcinoma **Preferred UMLS Name:** Metastatic Clear Cell Renal Carcinoma **Semantic Type:** Neoplastic Process

Disease Definition: A clear cell renal cell carcinoma that has spread extensively to other anatomic sites or is no longer responding to treatment. **MeSH Code:** C015797 **SNOMED ID:** 770177002 **SNOMED Hierarchy:** 186641006|Clear cell renal cell carcinoma|Clear cell renal cell carcinoma (disorder) **Medical Methodology:** TKI-sparing combination immunotherapy + HIF-2 α inhibition

Optimization Target: First-line (1L) advanced treatment response

13. Study Procedures & Methodology

Management Pathway: Standard oncology clinical pathway for advanced ccRCC **Education Protocol (HCP):** Protocol training and targeted AE management (e.g., recognizing irAEs, anemia, and hypoxia) **Education Protocol (Patient):** Study drug administration, side-effect reporting, and self-monitoring **Quality Audit**

Mechanism: Centralized imaging review (BICR) and routine source data verification

14. Observation & Follow-Up Schedule

Total Duration: Up to ~67 months (Estimated for Phase III) **Onsite**

Visit Cadence: Per standard treatment cycle for IV

immunotherapies **Remote Monitoring Frequency:** As needed for safety follow-up **Checklist Review Interval:** Tumor imaging assessment

routinely (e.g., every 8-12 weeks) based on RECIST 1.1

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					
Statistician	[Name]	_____	[DD-MMM-YYYY]					